



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : شرين نبيل محمد امين

العنوان : -----

الحى / المدينة : حلوان

المحافظة : حلوان

تاريخ تسجيل الإستمارة : 10-10-2026

رقم البطاقة : 28102200107325 رقم التليفون : 01140171222

العمر (تاريخ الميلاد) : 20-02-1981 المهنة : بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____

المقرر بما فيه : _____

التوقيع : (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة

قسم العلاج :
الأورام

[11129]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني

الاسم : د/ندى حسن

الاسم : د/نرمين رمضان

الوظيفة : _____

الوظيفة : _____

الوظيفة : _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة

رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / ----- / د. / ----- / د. / -----

قرار اللجنة

التشخيص النهائي : _____

خطة العلاج : _____

المستشفى : _____

قسم : _____

جهة العلاج : _____

نفقات : _____

ملاحظات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: شرين نبيل محمد امين
National ID: 28102200107325



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



٢٠٢٣/١١ ٢٨١.٢٢.٠١.٧٣٢٥

حاصله على بكالوريوس تجارة

أنثى مقيمة متزوجة
حسن سيد سليمان مندي

البطاقة سارية حتى ٢٠٢٠/١١/١١



DMS
مامو جريم



جمهورية مصر العربية
بطاقة تحقيق الشخصية

شربين
نبيل محمد امين
١٤ اش عامر-حكر التبين البحرى
التبين - القاهرة

١٩٨١/٠٧/٢٠

٢٨١.٢٢.٠١.٧٣٢٥

KG5465525



Ministry of Health and Population
Kingdom of Saudi Arabia

الوزارة العامة للصحة والسكان
Ministry of Health and Population



A-Patient ID:

1- Name: مريم نوري محمد أمين

Sex:

MDT form

3- Medical number:

Female

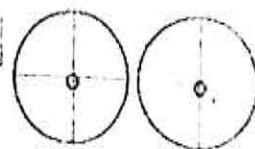
83145928

Old Med:

2096/2023

2-IO number: 28102200107325

Patient Age: 42 Y



Family history:

-VE

Phone:

01152895881

BMI:

Marital status:

NRS Score:

GROUP:

REASON OF MDY: NEW

Co morbidity:

FREE

Menopausal status:

PREMENOPAUSAL

Date of MDY:

2023-06-25

Clinical data					
Breast Imaging:			Mets Workup:		
			PETCT-->OPERATIVE BED RT BREAST HYPERMETABOLIC 3X2.5 SUV MAX 11 METABOLIC ACTIVE LN AND OSSEOUS LT 9TH RTB		
BX:	ER:	PR:	Her 2 Neu:	SIH:	KI67:
IDC	6/8	6/8	3		10
RT GII					
T					
H					
M					
Sonomammography					
RT BREAST SEVERAL IRREGULAR DENSE MASSES,,SPECULATED MASS 40X26 BIRADSS					
LT BREAST MASS BRADS 3					
PATIENT HISTORY					
HISTORY OF RT BREAST SINCE 2010 T2NOMO					
TRIPLE +VE RT BCS,,RECEIVED CHEMO,RADIO +HORMONAL TTT FOR 5YRS					
MDT OUTCOME & PLAN					
TAXOTER/ HERCEPTIN 3 CYCLES, ZOLADEX, ZOMETA (3 MONTHS) THEN REASSESS ADD					
MEDICAL ONCOLOGY		SIGNATURE			
SURGERY		RADIOLOGY		PATHOLOGY	
Psychiatric		Nutrition			

Name: شيرين نبيل محمد			
MR. : 1154			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
45Y PREM LOCALLY RECURENT RT BREAST CANCER WIH OLIGOMETTS TO BONE LEFT 9TH RIB SINCE 6/2023IDC 2 ER6/8 PR 6/8 HER2 3+ KI 10% HISTORY 2010 RT BCS REC ADJ CHEMO RADIO HORMONAL TILL 6/2023 METS AND LOCAL RECURRENCE REC TAXOTERE HERCEPTIN IV ST ARIMIDEX AND ZOLADEX FROM 3/2024 NOW ON HERCEPTIN FROM MERSAL CTS 11/12/2025 RD OF RT BREAST LESION SD OF LEFT 9TH RIB BS6/2025 ABNORMAL TRACER UPTAKE AT LEFT 9,10 RIBS FOR RAD CORR FOR ZOLADEX ARIMIDEX ZOMETA FOR MDT AS PT IS OLIGOMETTS for zometa ...after labs ...zoladex 10.8 ... anastrazol 1x1x90(3m) +supportive (1x1x90)(3m) cts 3/2026 sd,new cts pending for zoladex zometa arimidex ,pt is rec herceptin from mersal bs 4/2026 stable left 9th rib cts 7/2026 illdefined rt breast mass almosrt sd rt ax In new for new sm relative came on AI+zoladex+ herceptin as previously advised by MDT no role for surgey receiving herceptin at mersal 29/8/2026 SM bo comparison 8/2026 : 4-5 o'clock malignant mass 31x19mm vs 40x2mm (sm 2023) to continue 3 more months AI zoladex 8.10.2026			
تحريرا في ,			
		التاريخ :	19-09-2026
		الوقت :	
		الطبيب المعالج :	dina makram
يعتمد ,			

Patient Name : الأمشاذي / شربين نبيل محمد	Request No. : 13002223
Patient No. : 28102200107325	Request Date : 11/06/2023
Path No. : SR-702/23	Verification Date : 14/06/2023
Sex : Female	Financial Category : 3033
Age : 42 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

- One paraffin block labeled by (A 3340) was received.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * IPT (Positive control showed appropriate reactivity):
- E-cadherin: Diffuse positive.
- ER: Moderate nuclear reaction in 50% of tumor cells (Allred score 6/8).
- PR: Moderate nuclear reaction in 50% of tumor cells (Allred score 6/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Slide revision:
Invasive duct carcinoma, Grade II.
IPT:

- E-cadherin: Diffuse positive.
- ER: Positive (score 6/8).
- PR: Positive (score 6/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 10% of tumor cells.

Verified by :
Dr.Marwa mahmoud

Patient Name : الأستاذة / شيرين نبيل محمد
Patient No. : 28102200107325
Path No. : SR-702/23
Sex : Female
Age : 42 Y
Referring Doc :

Request No. : 13002223
Request Date : 11/06/2023
Verification Date : 14/06/2023
Financial Category : فئة الدولة
Department : Out Patient
Registered by : marwa mohamed

Pathology Report

Signature
Prof. Dr. Gamal Abdelaty
Dr. Walaa Ghonaim
Dr. Menna Nabil

Verified by :
Dr. Marwa mahmoud

Printing Date: 18/06/2023 14:08:13

Page 2 of 2

Printing By : marwa mahmoud

Patient Name: Sherine Nabil Mohamed Ameen	Date : 27/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 44 Year(s)
Medical ID: 20962023	Pat. ID: 53680

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

**Compared to last study dated 01-12-2024, The current
study shows :**

- **Chest:**
 - **Stationary course regarding the :**
 - Right breast irregular speculated soft tissue lesion with overlying skin thickening measuring 2.9x2 cm.
 - Left breast outer quadrant soft tissue lesion 1.3x1.1 cm
 - Small right internal mammary lymph node
 - No sizable pulmonary nodules, masses or cavitary lesions
 - No pathological hilar or mediastinal lymph nodes.
 - No evidence of pleural effusion, pleural thickening or masses seen.
 - Average cardiac size. No pericardial effusion detected.
 - Normal CT appearance of the thoracic aorta and the great vessels.
- **Abdomen and pelvis:**
 - The liver is average in size with hypodense homogenous CT texture. No definite focal lesions or contour irregularities.
 - Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
 - No intra-hepatic or extra-hepatic biliary radicles dilatation.
 - Normal size & parenchymal texture of the spleen with no focal lesions seen.
 - No pathological abdominal or pelvic lymph nodes.
 - Normal size, parenchymal thickness & density of both kidneys with , no back-pressure changes.
 - Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
 - Average filling of the urinary bladder with no filling

Patient Name: Sherine Nabil Mohamed Ameen	Date : 27/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 44 Year(s)
Medical ID: 20962023	Pat. ID: 53680

- IUD is noted in endometrial cavity.
- Bilateral pelvic phleboli.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The scanned skeleton:
 - Stable Left 9th lateral aspect expansile sclerotic osseous lesion with no sizable soft tissue component.
 - No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up study shows overall stationary course.

Dr. Ahmed Saeed Ali

Dr. Walid Mohamed, M.D.



Imaging report



كايرو سكان
CAIRO SCAN
Imaging & Lab

Name : Mrs. Shrin Nabil Mohamed Amin

Date : 16/11/2023

Code : R2899750

Room Name : Pet - CT - MA

Follow Up F-18 Fluorodeoxyglucose (FDG) PET/CT of the whole body:

Clinical History:

- Female patient with history of bilateral breast cancer, underwent right partial mastectomy, received radio and chemotherapy, hormonal and targeted therapy. The patient is referred for follow up.

Procedure:

- Dose/Radiopharmaceutical: F-18 Fluorodeoxyglucose (FDG) was administered via a peripheral upper limb vein.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- A low-dose non-contrast CT images for attenuation correction and an anatomic localisation were obtained followed by PET images from the vertex of the skull to the thighs.
- A diagnostic post-contrast CT examination of the same regions was performed after IV administration of non-ionic contrast.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- The Standardised Uptake Value, maximum variant (SUV max) was calculated within regions of interest whenever needed.

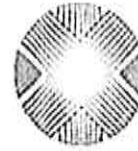
Findings

Compared to the last study dated 05/06/2023; the current one shows:

CURRENT	PREVIOUS
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Imaging report



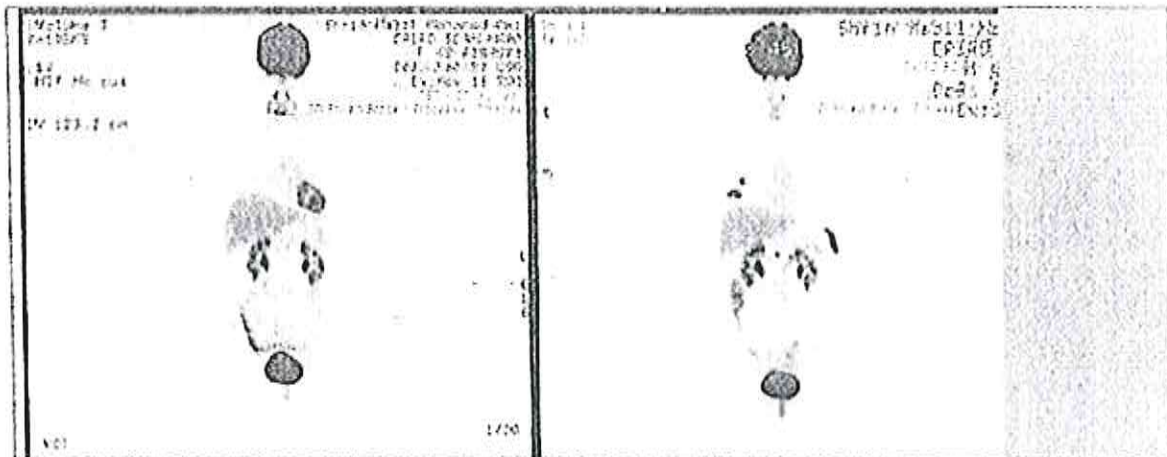
كايرو سكين
CAIRO SCAN
Radiology & PET

Name : Mrs. Shrin Nabil Mohamed Amin

Date : 16/11/2023

Code : R2899750

Room Name : Pet - CT - MA



Head and Neck:-

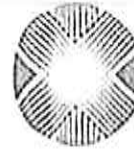
- The brain showing no gross lesions on CT, with no abnormal focal metabolic activity detected.
- No pathologically enlarged or metabolically active cervical lymphadenopathy.
- Physiologic radiotracer uptake is demonstrated at the oropharynx, larynx, thyroid and salivary glands.
- Normal FDG uptake pattern is seen at the soft tissues of the neck.

Chest:-

- Size and number and metabolic regression of previously noted metabolically active right breast parenchymal multicentric nodular spiculated soft tissue lesions, largest measuring about 2.3x1.8 cm at maximal diameters with SUVmax 3.4 compared to 3 x 2.5 cm with SUVmax 11.6 in the previous study. Still noted associated skin thickening reaching up to 6 mm maximal thickness.
- regression of previously noted metabolically active soft tissue lesion seen at lateral quadrant of left breast parenchyma measuring about 1.5x0.9 cm with SUVmax 2- maximal diameters compared to 2.3 x 2 cm with SUVmax 3.6 in the previous study.
- Resolution of previously noted metabolically active right subpectoral and right internal mammary lymph nodes at current study.
- No sizable metabolically active pulmonary nodules or other parenchymal abnormalities demonstrated.
- No pleural or pericardial collections.



Imaging report



كايرو سكان
CAIRO SCAN
Imaging & Lab

Name : Mrs. Shrin Nabil Mohamed Amin

Date : 16/11/2023

Code : R2899750

Room Name : Pet - CT - MA

- Normal tracer biodistribution is noted at the myocardium.

Abdomen and Pelvis:

- Mildly enlarged liver measures about 16 cm in MCL, showing homogenous fatty attenuation with smooth outlines.
- No focal lesions or areas of abnormal enhancement noted.
- Normal appearance of the spleen, pancreas, kidneys, suprarenals, aorta, IVC and pelvic organs (IUCD is noted).
- Metabolic resolution of the metabolically active retrocrural, mesenteric and right para-aortic lymph nodes at today's study.
- No ascites.
- Normal FDG activity is seen at the gastrointestinal and genitourinary systems, with no striking focal hypermetabolic lesions detected.

Musculo-skeletal:

- Marked metabolic regression of the hypermetabolic osteolytic destructive bony lesion is seen implicating the lateral aspect of the left 9th rib with resolution of the extraosseous soft tissue component showing current SUVmax3.1 compared to SUVmax14.6 in the prior study.

Impression:-

Follow up PET CT study showing:

- Regression of the metabolically active bilateral breast lesions and the metabolically active osseous (left 9th rib) deposit
- Resolution of the metabolically active metastatic nodal (right subpectoral , right internal mammary and abdominal) deposits
- No new gross hypermetabolic lesions detected at the rest of the scanned body.

Best Regards,

Dr. Iman Abd-ELAdel M.D

Prof.Dr. Maha Abd Elkareem
Professor of Nuclear Medicine
Kasr Al Ainy, Cairo University

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg